Magnet Status in Nurse Practitioners' Care Assessments and Hospital Recommendations

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Background: Nurse practitioners (NPs) are a growing segment of the acute care hospital workforce, yet little is known about how organizational factors, such as Magnet designation, influence their workplace experiences.

Objectives: We examined whether employment in a Magnet hospital was associated with NPs' strong recommendation of their hospital and whether this relationship was explained by NPs' assessments of care quality, patient safety, and care transitions.

Methods: In this cross-sectional study, we analyzed data from 774 NPs across 425 US hospitals using the 2024 Penn Nurses4All Survey and the American Hospital Association Annual Survey. The independent variable was employment in a Magnet hospital versus a non-Magnet hospital. The dependent variable was NPs' strong recommendation of their hospital to family and friends. Mediating variables included NPs' assessments of nursing care quality, patient safety, and confidence in patient care transitions. Mediation analysis using weighting estimators was conducted to adjust for confounding variables and to evaluate the extent to which the hypothesized mediators explained the relationship between Magnet status and hospital recommendation.

Results: NPs employed in Magnet hospitals were 15% more likely to strongly recommend their hospital compared with those in non-Magnet hospitals. The natural indirect associations through stronger quality of nursing care, patient safety grade, and confidence in care transitions accounted for ~13.3%, 26.6%, and 40% of the total association, respectively. The natural direct association remained positive and significant, suggesting that Magnet employment is associated with hospital recommendation among NPs independently of quality and safety assessments.

Discussion: NPs in Magnet hospitals are more likely to endorse their hospital, partially due to more favorable assessments of care quality and safety. This supports Magnet's aim to attract and retain registered nurses and suggests the benefit extends to NPs. Health care leaders should apply Magnet principles to strengthen NP engagement, development, and retention.

Key Words: Magnet hospitals, nurse practitioners, hospital recommendation, quality of health care, patient safety

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Nurse practitioners (NPs) represent one of the fastest-growing segments of the health care workforce in the United States (U.S. Bureau of Labor Statistics, 2025). Although national data indicate that most NPs practice in primary care (American Association of Nurse Practitioners, 2025), hospital executives are increasingly recognizing the value of integrating NPs into inpatient care teams (Reed & Ross, 2025). NPs in hospitals manage acutely ill patients with complex, evolving medical conditions. Their responsibilities often include conducting comprehensive assessments, making differential diagnoses, performing advanced clinical procedures, and coordinating multidisciplinary care plans (Hoffman & Guttendorf, 2017; Kleinpell et al., 2019). Emerging research has demonstrated that hospital-based NPs contribute significantly to the delivery of high-quality, cost-effective care in a variety of inpatient settings (Aiken et al., 2021; Woo et al., 2017). Despite the rapid expansion of this workforce (Auerbach et al., 2020) and their growing clinical contributions, there remains a significant gap in the literature regarding the work environments of acute care hospital-based NPs (Yu, Connell, et al., 2025).

The Magnet Recognition Program, administered by the American Nurses Credentialing Center (ANCC), is a designation awarded to health care organizations that demonstrate excellence in nursing practice, high-quality patient care, and innovations in professional nursing (American Nurses Credentialing Center, 2025a). Although the Magnet Recognition Program has been extensively studied in relation to staff nurse outcomes (Rodriguez-Garcia et al., 2020), its effect on NPs, particularly those

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working in hospital settings, remains largely underexamined. As NPs continue to expand their presence across inpatient care and take on increasingly complex clinical roles, it is critical to understand whether the benefits associated with Magnet designation extend to this essential segment of the nursing workforce.

This study is guided by the model of quality of health care (Donabedian, 1980), a widely used framework that conceptualizes health care quality through three interrelated components: structure, process, and outcome. Structure refers to the organizational environment in which care is delivered, including staffing, resources, and institutional support. Process involves the actual delivery of care, and outcome reflects the consequences of care provided.

In this study, the structural component is represented by the Magnet designation. The process component includes NPs' assessments of nursing care quality, patient safety, and care transitions, key indicators of daily care delivery. The outcome of interest is NPs' strong recommendation of their hospital to family and friends. Indirect measures such as provider preferences and behaviors have been recognized as valid indicators of care quality (Donabedian, 2005). This recommendation metric, commonly used in patient satisfaction surveys for hospital care (Centers for Medicare & Medicaid Services, 2025), serves as a well-established proxy for provider engagement and perceived organizational quality, with strong concordance between provider and patient assessments (Aiken et al., 2012).

Guided by this framework, we hypothesize that NPs in Magnet hospitals are more likely to strongly recommend their hospital, partly due to more favorable assessments of care quality and safety. Since Magnet designation emphasizes professional practice, innovation, and supportive work environments (American Nurses Credentialing Center, 2025a), these structural features may enhance care processes, thereby increasing NPs' organizational endorsement. We hypothesized the following:

1. NPs employed in Magnet hospitals will be more likely to strongly recommend their hospital compared with those in non-Magnet hospitals.
2. NPs in Magnet hospitals will report more favorable assessments of nursing care quality, patient safety, and care transitions than those in non-Magnet hospitals.
3. NPs' assessments of care quality and safety will partially explain the association between employment in a Magnet hospital and a strong hospital recommendation.

METHODS

Study Design and Data Sources

In this cross-sectional study, we integrated data from the 2024 Penn Nurses4All Survey, the 2023 American Hospital Association (AHA) Annual Survey, and the list of Magnet-designated organizations from the ANCC. The Penn Nurses4All Survey assessed nurse job outcomes and work environments across 10 U.S. states (CA, FL, IL, LA, NJ, NM, NY, OR, PA, WA). In 2023, nurses in nine states were invited via email using licensure lists from state boards. In Pennsylvania, due to the unavailability of email addresses, a random sample of approximately 30% of the licensure list was selected and invited to participate via mail. Nurses reported their hospital from a provided list or entered the name manually if it was not listed. Hospital characteristics were obtained from the 2023 AHA Annual Survey. This study was approved by the University of Pennsylvania Institutional Review Board (Approval No. 850227).

Study Sample

In the Nurses4All Survey, 1,393 respondents identified as NPs employed in hospital settings. Hospital-based NPs were defined as respondents who confirmed their NP role and identified a hospital as their primary practice setting; employment arrangement (hospital-employed vs. contracted) was not collected. Because the primary aim of this study was to examine the relationship between hospitals' Magnet status and NPs' hospital recommendations, it was essential to identify each respondent's hospital to determine its Magnet status. Therefore, we excluded 591 NPs who reported working in hospitals but did not provide the name of their hospital. We further excluded 28 NPs who had missing values on key study variables. The final analytic sample included 774 NPs working in 425 hospitals, including 126 Magnet and 299 non-Magnet hospitals.

Study Variables

Dependent Variable: NPs' Strong Recommendation of Their Hospital

NPs' likelihood to recommend their hospital to family or friends was measured using a single item: "Would you recommend where you work to your family and friends if they needed health care?" This question has been widely validated across international settings, demonstrating strong concordance between nurses' and patients' responses regarding which hospital provides good care (Aiken et al., 2012). Responses were captured on a 4-point Likert scale, with options, including "1 = *definitely yes*," "2 = *probably yes*," "3 = *probably not*," and "4 = *definitely not*" (Aiken et al., 2012). For analysis, responses were dichotomized, with 1 representing *definitely yes* and 0 combining *probably yes*, *probably not*, and *definitely not*. This binary coding approach was used to isolate strong, unequivocal endorsements of the hospital (*definitely yes*) from more uncertain or negative responses.

Independent Variable: Employment in Magnet Hospitals

The independent variable was a binary indicator of whether the NP was employed in a Magnet hospital. This variable was coded as 1 if the NP reported working in a Magnet hospital, based on the ANCC's list of Magnet-designated organizations as of 2024 (American Nurses Credentialing Center, 2025b), and 0 if they were employed at a non-Magnet hospital.

Mediating Variable: Quality and Safety Assessments

Quality of nursing care was assessed using a single item that has been strongly associated with objective patient outcomes, including mortality (McHugh & Stimpfel, 2012; Smeds-Alenius et al., 2016). The question, "In general, how would you describe the quality of nursing care delivered to patients in your work setting?" was answered on a 4-point Likert scale, with responses, including "1 = *excellent*," "2 = *good*," "3 = *fair*," and "4 = *poor*." For analysis, responses were dichotomized, with *excellent* and *good* coded as 1, and *fair* and *poor* coded as 0, thereby

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distinguishing favorable assessments of care quality from less favorable ones.

Patient safety was measured using a single-item question: "Please give your current practice setting an overall grade on patient safety." Responses were recorded on a 5-point Likert scale: "1 = *excellent*," "2 = *good*," "3 = *acceptable*," "4 = *poor*," and "5 = *failing*." For analysis, responses of *excellent* and *good* were coded as 1, while *acceptable*, *poor*, and *failing* were coded as 0. This binary coding approach was used to isolate strong endorsements of patient safety culture from more neutral or negative assessments.

Confidence in care transitions was assessed using a single-item question: "How confident are you that your patients and their caregivers can manage their care after discharge?" Responses were recorded on a 4-point Likert scale: "1 = *very confident*," "2 = *confident*," "3 = *somewhat confident*," and "4 = *not at all confident*." For analysis, responses of *very confident* and *confident* were coded as 1, indicating high confidence in care transition, while all other responses were coded as 0.

Covariates: NP Professional and Hospital Characteristics

We included various NP professional and hospital characteristics in our models to correct for the potential imbalances between Magnet and non-Magnet hospitals. Professional characteristics among NPs were selected for their potential to confound the relationship between Magnet employment and NPs' likelihood of recommending their hospital. These included years of experience, the highest nursing degree, and the unit setting. Years of experience, for example, might be associated with both Magnet employment and assessments of hospital quality and recommendations, as more experienced NPs may be more selective and have higher expectations. This variable was treated as continuous. The highest nursing degree, categorized as *master's* or *doctoral*, reflects professional training that may shape both employment opportunities and assessments of care quality. Unit setting may also confound the relationship, as certain units, such as ICUs or specialty services, differ in organizational resources, patient acuity, and support structures, which can influence care quality assessments and recommendations. Unit setting was initially reported in 20 closed-ended categories (e.g., various hospital units or specialty services), along with a write-in option. After reviewing all write-in entries, unit settings were consolidated into three groups: critical care units, noncritical-care units, and specialty services.

Hospital characteristics included teaching status, size, advanced service capacity, metropolitan location, system membership, and ownership. These factors were considered potential confounders because they are known to be associated with both the likelihood of hospitals achieving Magnet status and the quality of nursing care (Mazurenko et al., 2017; Yu, Brom, et al., 2025). Teaching status was based on the trainee-to-bed ratio: *non-teaching* (no trainees), *minor teaching* ($\leq$1:4), and *major teaching* ($>$1:4), a more accurate measure than university affiliation (Ayanian et al., 1998). Hospitals offering open-heart surgery or major transplants were classified as *high-capacity* (Hartz et al., 1989). Size was categorized as *small* ($\leq$100 beds), *medium* (101–250), and *large* ($>$250) (Aiken et al., 2002). Ownership types included *not-for-profit*, *for-profit*, and *federal*. Metropolitan status was defined using Core Based Statistical Area (CBSA) codes (Congressional Research Service, 2024), dichotomized into *metropolitan* and *non-metropolitan* (combining rural and micropolitan). System membership was coded as a binary variable: 0 for *no system membership* and 1 for *system membership*.

Statistical Analysis

All data analyses were conducted using R version 4.4.1. We began with bivariate analyses to examine unadjusted differences across Magnet and non-Magnet hospitals in key study variables, including NP professional characteristics, hospital characteristics, quality and safety measures, and NPs' likelihood of strongly recommending their hospital.

To assess whether NPs' evaluations of care quality and safety explain the observed association between Magnet hospital employment and strong hospital recommendation, we conducted a mediation analysis using weighting estimators (Coffman et al., 2023) implemented through the *twangMediation* R package (Coffman et al., 2022). This approach is appropriate for models involving a binary independent variable (Magnet employment), binary mediators (strong quality of nursing care, patient safety grade, and confidence in care transitions), and a binary dependent variable (strong hospital recommendation).

The mediation analysis involved two primary steps. First, we estimated the total effect, capturing the overall association between Magnet employment and the outcome. Second, we decomposed this total effect into the natural direct effect (NDE) and the natural indirect effect (NIE) to determine the extent to which the association is explained by the mediating variables. In Step 1, we estimated propensity scores using generalized boosted models via the *twang* R package (Ridgeway et al., 2024) to create inverse probability weights, referred to as total effect weights, that balance covariates between NPs in Magnet and non-Magnet hospitals. These weights allowed us to estimate the average treatment effect of Magnet employment on hospital recommendation. In Step 2, we computed an additional set of propensity score weights, referred to as cross-world weights (Nguyen et al., 2023), to estimate the NDE and the NIE. These weights simulate hypothetical ("cross-world") scenarios in which the mediator is held constant while Magnet status varies (Schuler et al., 2024). The NIE reflects the portion of the association attributable to the mediator (e.g., quality of care assessment), while the NDE represents the remaining association between Magnet status and the outcome (Coffman et al., 2022; Schuler et al., 2024). This decomposition allowed us to determine whether the relationship between Magnet employment and hospital recommendation is primarily explained by NPs' assessments of care quality and safety, or by other mechanisms.

In Step 1, propensity scores were estimated using generalized boosted modeling, a flexible, nonparametric alternative to logistic regression (Lee et al., 2010), via the *ps* function in the *twang* package (Ridgeway et al., 2024), with Magnet employment regressed on NP professional and hospital characteristics. The Kolmogorov–Smirnov (KS) statistic was used to assess covariate balance, with values above 0.10 indicating potential imbalance (McCaffrey et al., 2013). These weights were then used in Step 2 within the *wgtmed* function in the *twangMediation* package to estimate the NIE and NDE, allowing for the decomposition of the total effect. The mediation analysis was conducted separately for each binary mediator. Missing data in covariates were

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addressed by constructing weights that balanced the distribution of missingness across Magnet and non-Magnet groups.

Among the 802 NPs who provided their hospital names, 28 had missing values in the mediating or outcome variables. Although the exact mechanism underlying the missing data was uncertain, there was no evidence to suggest it was associated with any observed variables. While missing data in covariates were handled through weighting, mediation models require complete data on mediators and outcomes (Coffman et al., 2022). Thus, we restricted our final analytic sample to 774 NPs with no missing data in these variables, while still including covariates with missingness. To assess robustness, we compared the mediation results from the complete-case sample ($n = 774$) with those from an imputed dataset ($n = 802$) and found no meaningful differences in statistical significance. Given the small proportion of missingness (3.5%), we proceeded with the sample of 774 NPs.

RESULTS

Table 1 presents unadjusted comparisons of NPs' assessments of nursing care quality, patient safety, and hospital recommendations by Magnet status. NPs in Magnet hospitals were significantly more likely to "definitely" recommend their hospital (46.1% vs. 35.2%, $p = .003$), rate patient safety as "excellent" or "good" (73.0% vs. 63.1%, $p = .005$), and express strong confidence in patients' ability to manage care after discharge (52.0% vs. 43.5%, $p = .024$).

Table 2 presents NP professional and hospital characteristics. On average, NPs had 11.6 years ($SD = 10.7$) of experience at their current hospital. Most held a master's degree (79.6%), followed by doctoral degrees (20.4%). NPs worked in critical care units (27.7%), noncritical-care units (42.9%), and specialty services (29.4%). Most were employed at large (72.5%) and metropolitan (94.0%) hospitals. The majority of NPs worked in teaching hospitals (80.1%), hospitals with high specialized service capacity (56.2%), those affiliated with a system (72.5%), and not-for-profit institutions (86.4%). Compared with NPs in non-Magnet hospitals, NPs in Magnet hospitals had longer tenure at their current hospital (M = 12.6 vs. 10.9; $p = .027$). They were also more likely to practice in large hospitals (89.6% vs. 58.6%; $p < .001$), metropolitan locations (99% vs. 90%; $p < .001$), teaching hospitals (87.6% vs. 74.1%; $p < .001$), facilities with high specialized service capacity (81.3% vs. 35.9%; $p < .001$), and not-for-profit hospitals (99% vs. 76.2%; $p < .001$).

Baseline NP professional and hospital characteristics before and after propensity score weighting are presented in SDC 1, Supplemental Digital Content 1, http://links.lww.com/NRES/A585. After weighting, all KS statistics were below 0.10, indicating adequate balance across confounding variables between Magnet and non-Magnet groups. Covariate balance for the natural direct effect (NDE) and the natural indirect effect (NIE) after applying cross-world weighting is presented in SDC 2 to 4, Supplemental Digital Content 1, http://links.lww.com/NRES/A585. Each SDC corresponds to one of the three mediators and demonstrates that the weighting approach successfully balanced all observed covariates between the comparison groups for each mediation pathway.

Table 3 shows direct and indirect associations between Magnet employment status, quality and safety assessments, and NPs' hospital recommendations. The total association estimate of 0.15 indicates that NPs employed in Magnet hospitals were 15% more likely to strongly recommend their hospital compared to those in non-Magnet hospitals (95% CI: 0.06–0.23). The natural indirect associations through quality of nursing care assessment, patient safety grade, and confidence in care transitions accounted for approximately 13.3%, 26.6%, and 40% of the total association, respectively. The natural direct association remained positive and statistically significant, suggesting that Magnet employment is associated with hospital recommendation among NPs independently of quality and safety assessments.

DISCUSSION

Our study is among the first to examine the effect of hospitals' Magnet status on NPs in hospital settings. Consistent with our hypotheses, NPs in Magnet hospitals were more likely to strongly recommend their hospital compared to those in non-Magnet hospitals, and their more favorable assessments of nursing care quality, patient safety,

TABLE 1. Assessments of Quality, Safety, and Hospital Recommendation Among Nurse Practitioners

	n (%)			
	Overall ($N = 774$)	**NPs in Magnet hospitals ($n = 319$)**	**NPs in non-Magnet hospitals ($n = 455$)**	***p***
Likelihood to recommend their hospital				.003
Definitely yes	307 (39.7)	147 (46.1)	160 (35.2)	
Definitely not, probably not, or probably yes	467 (60.3)	172 (53.9)	295 (64.8)	
Quality of nursing care				.099
Poor or fair	149 (19.3)	52 (16.3)	97 (21.3)	
Excellent or good	625 (80.7)	267 (83.7)	358 (78.7)	
Patient safety				.005
Failing, poor, or acceptable	254 (32.8)	86 (27.0)	168 (36.9)	
Excellent or good	520 (67.2)	233 (73.0)	287 (63.1)	
Confidence in care transitions				.024
Very confident or confident	364 (47.0)	166 (52.0)	198 (43.5)	
Somewhat confident or not at all confident	410 (53.0)	153 (48.0)	257 (56.5)	

Note. NP = nurse practitioner.

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TABLE 2. Nurse Practitioner Characteristics by Magnet Hospital Status

	Overall ($N = 774$)	NPs in Magnet hospitals ($n = 319$)	NPs in non-Magnet hospitals ($n = 455$)	p
Years of experience in the current hospital, *M (SD)*	11.6 (10.7)	12.6 (11.0)	10.9 (10.4)	.027
	n (%)	*n* (%)	*n* (%)	
Highest degree				.713
Master's degree	518 (79.6)	218 (80.4)	300 (78.9)	
Doctoral degree	133 (20.4)	53 (19.6)	80 (21.1)	
Unit setting				.377
Critical care unit	212 (27.7)	95 (30.0)	117 (26.1)	
Noncritical-care unit	328 (42.9)	136 (42.9)	192 (42.9)	
Specialty service	225 (29.4)	86 (27.1)	139 (31.0)	
Hospital size				< .001
Small (up to 100 beds)	50 (7.5)	2 (0.7)	48 (13.0)	
Medium (101–250 beds)	134 (20.0)	29 (9.7)	105 (28.4)	
Large (over 250 beds)	485 (72.5)	268 (89.6)	217 (58.6)	
Metropolitan location				< .001
Metropolitan	629 (94.0)	296 (99.0)	333 (90.0)	
Non-metropolitan	40 (6.0)	3 (1.0)	37 (10.0)	
Teaching status				< .001
None	133 (19.9)	37 (12.4)	96 (52.9)	
Minor	257 (38.4)	82 (27.4)	175 (47.3)	
Major	279 (41.7)	180 (60.2)	99 (26.8)	
Specialized service capacity				< .001
High capacity	376 (56.2)	243 (81.3)	133 (35.9)	
Low capacity	293 (43.8)	56 (18.7)	237 (64.1)	
System membership				.999
Yes	485 (72.5)	217 (72.6)	268 (72.4)	
No	184 (27.5)	82 (27.4)	102 (27.6)	
Ownership				< .001
Not-for-profit	578 (86.4)	296 (99.0)	282 (76.2)	
For-profit	31 (4.6)	0 (0)	31 (8.4)	
Federal	60 (9.0)	3 (1.0)	57 (15.4)	

Note: *M* = mean; NP = nurse practitioner; *SD* = standard deviation.
The sum of row values may not equal the column total because of missing data.

and care transitions partially explained this association. These results suggest that Magnet designation may foster conditions that enhance care processes as assessed by NPs, which in turn contribute to higher levels of organizational endorsement.

NPs' likelihood of recommending their Magnet hospital mirrors patterns observed among patients. In 2023, publicly reported Hospital Consumer Assessment of Healthcare Providers and Systems (HCAHPS) data, a standardized survey that measures patient satisfaction with hospital care, showed that patients were significantly more likely to recommend Magnet hospitals than non-Magnet hospitals (Yu, Brom, et al., 2025). Among the various HCAHPS domains, patient satisfaction with care transitions was notably higher in Magnet hospitals, aligning with our findings that NPs in Magnet hospitals expressed greater confidence in patients' ability to manage their care after discharge (Yu, Brom, et al., 2025). This concordance reinforces the validity of provider recommendations as a meaningful indicator of organizational quality and underscores potential organizational benefits of investing in Magnet standards, both in enhancing patient experience and in improving provider engagement.

Our study contributes to the growing body of literature suggesting that the Magnet Recognition Program, while historically evaluated through the lens of staff nurses (Anderson et al., 2018), may offer similar advantages for NPs, whose scope of practice and roles differ significantly from those of staff nurses (Hoffman & Guttendorf, 2017;

TABLE 3. Direct and Indirect Associations Between Magnet Status, Quality and Safety, and Nurse Practitioners' Hospital Recommendations

	Quality of Nursing Care			Patient Safety Grade			Confidence in Care Transitions		
	Estimate	*SE*	[95% CI]	Estimate	*SE*	[95% CI]	Estimate	*SE*	[95% CI]
Natural indirect association	0.02	0.01	[0.01–0.03]	0.04	0.01	[0.02–0.06]	0.06	0.01	[0.03–0.08]
Natural direct association	0.13	0.04	[0.05–0.22]	0.10	0.04	[0.02–0.19]	0.09	0.04	[0.01–0.18]
Total association	0.15	0.04	[0.06–0.23]	0.15	0.04	[0.06–0.23]	0.15	0.04	[0.06–0.23]

Note. CI = confidence interval; *SE* = standard error.

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Kleinpell et al., 2019). Given the expanding clinical role of hospital-based NPs in inpatient care delivery, our results underscore the importance of more comprehensively including NPs in future Magnet evaluations and tailoring organizational supports to reflect their scope of practice, professional needs, and contributions to hospital performance. In addition, Magnet hospitals should also invest in targeted organizational interventions that address the distinct needs of NPs. While many Magnet initiatives have historically focused on improving work environments for staff nurses, NPs often occupy hybrid roles that span both nursing and medicine, leading to unique challenges related to professional visibility, role clarity, and inclusion in decision-making. For example, NPs may lack access to formal leadership structures tailored to their role and experience less autonomy in institutional decision-making (Yu, Connell, et al., 2025).

These challenges can contribute to adverse job outcomes among hospital-based NPs. A recent study examining the well-being of advanced practice registered nurses (APRNs), including NPs, in Magnet hospitals found that APRNs reported higher levels of burnout than physicians, and nearly one-third expressed an intention to leave their current positions (Kennedy et al., 2025). To improve NP work experience, Magnet and other hospitals should adopt targeted, evidence-based strategies, including implementing effective transition-to-practice programs; developing NP-specific career and leadership pathways; establishing NP executive leadership roles and clear reporting lines; ensuring meaningful recognition and fair compensation; intentionally including NPs in organizational decision-making; maintaining safe patient ratios and adequate support staff; optimizing professional practice by setting service-specific workload norms and clarifying roles; and fostering psychologically safe team cultures (Yu, Connell, et al., 2025).

These findings also highlight the need for Magnet hospitals to move beyond structural indicators of excellence and to implement dedicated strategies that promote NP well-being, support role-specific leadership, and ensure inclusive practice environments tailored to the advanced practice workforce. Key components of the Magnet Model (American Nurses Credentialing Center, 2025c) offer a foundation for this effort. For example, the Transformational Leadership domain could be expanded to ensure that NPs have access to leaders who understand the scope and effect of advanced practice roles, ideally through standardized NP-led leadership models. Under the Structural Empowerment domain, hospitals can support NPs by promoting shared governance structures that include NP voices, ensuring access to professional development opportunities, and tracking NP-specific metrics such as job satisfaction, retention, and burnout.

Limitations

Several limitations should be acknowledged. First, our cross-sectional design precludes causal inference. Second, our sample may not fully represent all hospital-employed NPs nationally, as it was drawn from 10 U.S. states. However, these states represent a diverse mix of geographic regions, hospital types, and health systems. Third, the use of single-item measures for key constructs, while validated, may not capture the full nuance of NPs' workplace experiences. Fourth, we excluded 591 NPs who did not report their hospital name. Bivariate analyses indicated that, compared with included NPs, these NPs had shorter tenure, were more likely to work in critical care, and were more often male, potentially limiting generalizability. Finally, we conducted three separate mediation models to examine the unique contribution of each binary mediator. The estimated proportions of the total association explained by each mediator should not be interpreted as additive, as these mediators are moderately correlated and may convey overlapping information. Consequently, the combined indirect association is likely smaller than the sum of the individual associations. Future research using joint or multiple mediator models may be warranted to better understand their collective explanatory power while accounting for shared variance.

CONCLUSION

NPs in Magnet hospitals are more likely to strongly recommend their hospital, and this association is partially explained by their more favorable assessments of nursing care quality, patient safety, and care transitions. Consistent with Magnet's founding purpose of attracting and retaining nurses, our results suggest these benefits may extend to NP attraction and retention, indicating that hospital-based NPs, whose roles differ significantly, also experience meaningful advantages in Magnet environments. As the NP workforce continues to expand in hospital settings, health care leaders should apply Magnet principles to support NP engagement, promote well-being, and strengthen workforce attraction and retention.

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